

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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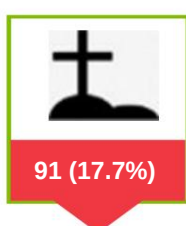
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°23/MVB_06/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	- Ituri (17/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara - North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha - South Kivu (1/34): Miti-Murhesa
Reporting date	June 6, 2026
Publication date	June 7, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data currently being harmonized

1. HIGHLIGHTS

Twenty **-seven (27) new confirmed cases, including 4 deaths**, were reported on June 6, 2026, in Ituri Province. These cases are distributed across the health zones of **Mongbwalu (18)**, **Rwampara (5)** and **Bunia (4)**.

Three **(3) confirmed** Ebola virus disease patients have been declared recovered, two (2) in the Bunia health zone (Ituri province) and one (1) in the Katwa health zone (North Kivu province). The total number of recoveries has thus increased from 9 to 12.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

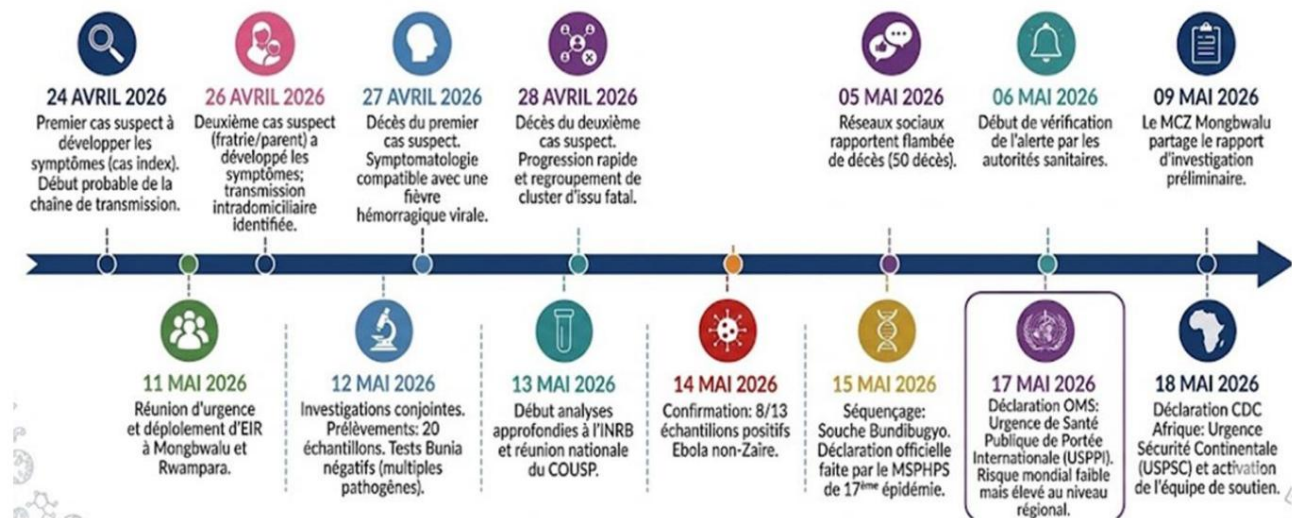


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 6, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	487	74	15.2%	17 out of 36 (47.2%)	27
North Kivu	25	16	64.0%	7 out of 34 (20.6%)	0
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	515	91	17.7%	25 out of 104 (24.0%)	27

- No new health zones were affected by the end of the day on June 6, 2026. 94.5% of confirmed cases are located in the province of Ituri, making it the epicenter among the 3 affected provinces. Although the overall case fatality rate in the affected provinces remains low, particularly in Ituri (15.2%), North Kivu province (64.0%) bears the heaviest burden. South Kivu province remains the least affected, with its last confirmed case dating back to May 26, 2026.
- The worrying lethality (64.0%) in the province of North Kivu can be explained by delays in care as well as the escape of confirmed cases from care facilities.

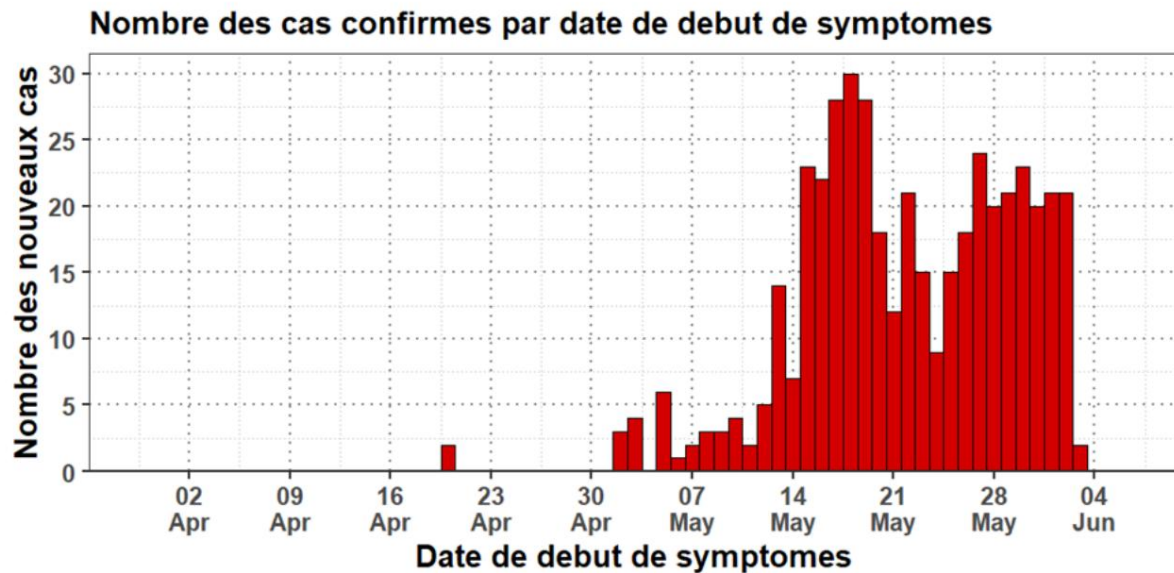
3.2 Distribution by health zone (Cumulative as of 06/06/2026)

- Ituri Province (487 cases):** Bunia (142), Rwampara (98), Mongbwalu (92), Nyankunde (24), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damas (3), Kilo (4), Nizi (5), Mangala (1), Aungba (1), Gety (1), Komanda (3), Lita (4), Logo (2). Ninety-four (94) additional cases are being ventilated.
- North Kivu Province (25 cases):** Katwa (11), Beni (5), Butembo (4), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time analysis (missing data from June 5, 2026) : A significant number of confirmed cases

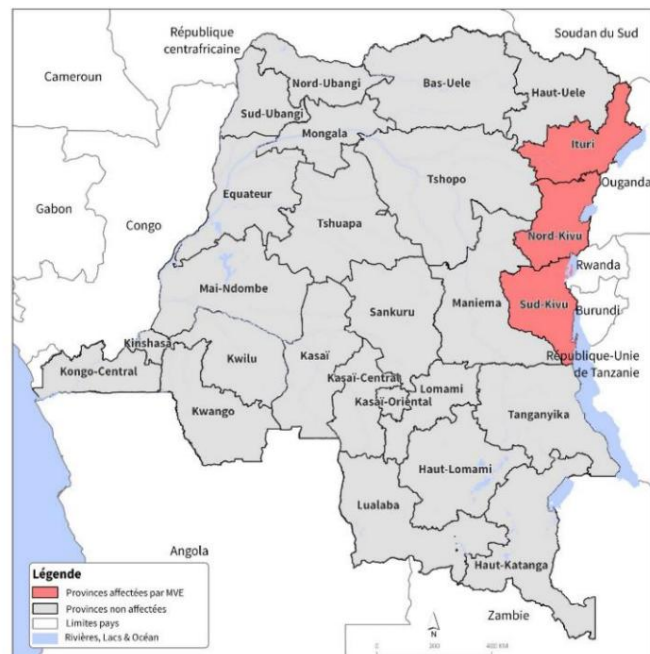
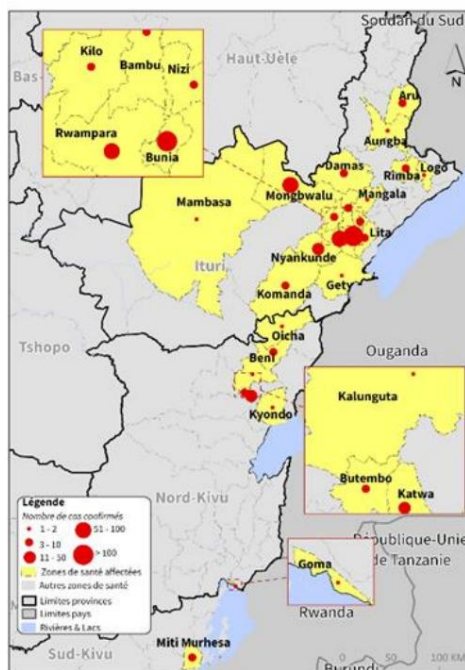
(cohort 1) was symptomatic in the period between May 14 and May 23, 2026, suggesting increased contamination from a probable common outbreak, with a peak observed on May 18, 2026.

Subsequently, a number of confirmed cases (cohort 2) began showing symptoms between May 25th and June 3rd, demonstrating the spread of the disease and potentially constituting a significant reservoir. An increase in cases may be recorded if appropriate measures are not implemented very quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 447.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 6, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 6, 2026

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI			
Bunia	142	14	9.9%
Rwampara	98	19	19.4%
Mongbwalu	92	25	27.2%
Nyankunde	24	1	4.2%
Bamboo	5	2	40.0%
Aru	3	1	33.3%
Kilo	4	1	25.0%
Nizi	5	0	0.0%
Mangala	1	0	0.0%
Damascus	3	0	0.0%
Aungba		0	0.0%
Gety		0	0.0%
Komanda	1	0	0.0%
Lita	1 3 4	0	0.0%
Logo	2	0	0.0%
Mambasa	2		50.0%
Rimba	3	10	0.0%
Other ZS (undisclosed data)	94	10	10.6%
Ituri subtotal	487	74	15.2%
NORTH KIVU			
Katwa	11	8	72.7%
Blessed	5	3	60.0%
Butembo	4	2	50.0%
Oicha	2	2	100.0%
Kalunguta	1	1	100.0%
Kyondo	1	0	0.0%
Goma			0.0%
North Kivu subtotal	1 25	0 16	64.0%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal		1	33.3%
TOTAL	3,515	91	17.7%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- **National / Ituri:**
- Continued support to affected provinces under the leadership of the IM and SGI members based in Bunia, Ituri province.
- Holding a meeting with the PTFs to guide the response according to a prioritization based on the analysis risks.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 6, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	134	208	3	345
Alerts investigated	111	208	3	322
Investigation rate	82.8%	100.0%	100.0%	93.3%
Alerts confirmed	86	30	1	117

Considering the data reported as of June 6, 2026, 345 alerts were raised, 322 (93.3%) were investigated, among which **117 suspected cases were recorded** in the 3 affected provinces. More than half (60.3%) of the notified alerts were located in North Kivu province, 38.8% in Ituri and 0.9% in South Kivu. Moreover, the lowest investigation rate (82.8%) was noted in the province of Ituri.

Table 3. Contact tracing of confirmed cases as of June 6, 2026

Province	Contacts under follow-up (n)	Contacts viewed	Follow-up rate (%)
Ituri	(n) 4,118 1,779		43.2%
North Kivu	699	577	82.5%
South Kivu	223	179	80.3%
Total	5,040	2,535	50.3%

- Routine field surveillance activities (reporting alerts, investigations, active searching, listing and monitoring contacts, retrospective review of cases and deaths, etc.)
- Thorough investigations around confirmed cases in affected health zones.
- In the health zones of Bunia (110) and Rwampara (100), 210 contacts have completed their 21 follow-ups days and were unloaded.

Update on checkpoint and point of entry (PoC/PoE) activities

- Closure of PoE Mahagi-Port
- Situation report at the Mudzibala PoC by the ICCN for the start of cabin construction

Indicators	Results
Proportion of activated PoCs	23.6% (13/55)
Number of travelers screened, % of	14,378
travelers screened, % of travelers	97.9%
refused screening, % of	2.1%
travelers who washed their hands, % of	98.9%
travelers who refused to wash their	1.1%
hands, % of travelers made aware of the risks	95.9%
Number of alerts notified	5
Percentage of alerts investigated within 24 hours	0%
Number of validated alerts	5
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 94 samples collected and analyzed (positivity 28.7%; n=27)
- **North Kivu:** 6 samples were collected and analyzed, none of which tested positive for Ebola virus disease. However, 193 results remain pending due to a lack of reagents.
- **South Kivu:** 6 samples received and analyzed in the laboratory, all negative for Ebola virus disease.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** 6/15 EDS (Emergency Diagnostic Sites) completed and 23 healthcare providers from 6 ESS (Emergency Health Services) briefed on Ebola virus disease (EVD) general information, bio-cleaning, and standard precautions. In the Aru Health Zone: 10 healthcare providers were briefed on standard precautions, the donning and doffing of PPE, waste management, and decontamination of the household of a confirmed case; 3 EDS carried out out of 5 alerts (2 postponements due to resistance from families). In Mongbwalu, 8 EDS carried out out of 9 alerts, decontamination of 7 ESS and briefing of the 33 service providers on hand washing and waste management.
- **North Kivu:** Risk assessment of CH Saint Joseph providers in AS Rughenda; PCI assessment with the scorecard tool: Clinique La vie des Hommes (27.5%), Disp Nya Kalunga Makuha (22.5%), CS Vuhika (51.1%) and CS Masuli (46.5%), hence an average score of 36.9%; decontamination of 2/6 premises of Horizon university clinics in the Goma health zone around two healthcare providers who tested positive.
- **South Kivu :** Support to the EIR for the Nyangezi investigation; evaluation of scorecards in two facilities in the Miti-Murhesa health zone (Chaoboka: 38% and Lwiro Centre Hospital: 43%) and development of improvement plans for these two facilities; support for the decontamination of the intensive care unit at the General Referral Hospital Katana; distribution of PCI inputs (100 handwashing kits, including 40 at ESS and 60 in the community, 40 decontamination kits in the Mitimurhesa and Katana health zones).

4.5 Holistic care

Table 4. Patient movement in healthcare facilities as of June 6, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	257	16	7	280
Total admissions (24h)	29	1	0	30
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	0	7
Non-cases /	13	1	0	14
Recovered / Escaped (Suspects/Conf.)	6	0	0	6
Total discharges	26	1	0	27
of patients in isolation (End of	260	16	7	283
Day), including confirmed	99	5	1	105
(NC+AC) and suspected cases	161	11	6	178

Mental and psychosocial health activities

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
Proportion of newly confirmed cases that benefited from SMSPS interventions	6 (22.2%)		
Proportion of confirmed cases under follow-up that benefited from SMSPS interventions	16 (20.0%)	3 (100.0%)	3 (100.0%)
Proportion of new suspected cases that benefited from SMSPS interventions	9 (31.0%)		2 (100.0%)
Proportion of suspected cases under follow-up that benefited from SMSPS interventions	18 (13.6%)	13 (86.7%)	3 (100.0%)
Proportion of laboratory results having been announced	43		
Including positive ones	14 (32.6%)		
Proportion of children separated in daycare who benefited from SMSPS interventions	9		3 (100.0%)
Proportion of children separated in the community who benefited from SMSPS interventions	0		6 (46.2%)
Number of accompanying persons at the CTE who benefited from SMSPS interventions	40	6	6
Number of PPLs who benefited from SMSPS interventions	15	9	6
Number of households and communities that benefited from SMSPS interventions	40	13	4
Proportion of patients who received Dignity Kits at the CTE	0	0	1 (100.0%)

4.6 Continuity of services.

- Nothing to report

4.7 CREC

- **North Kivu:** Briefing of 19 community leaders and CODESA members from the Kirima health area on community-based surveillance and appropriation of Ebola prevention measures; briefing of Management Committee members in the health zones on CREC for appropriation of Ebola prevention measures; conducting 7 educational talks on community-based surveillance and Ebola prevention measures, reaching 361 people (175 women and 186 men), all members of mutuals and youth associations, motorcyclists, and prayer groups in the Mbimbi, Mabasele, Mambabeka and Pakanza health areas; 2,454 people sensitized during Home Visits (HV), including 1,232 men and 1,222 women, on Ebola prevention measures.
- **South Kivu:** Mass awareness-raising of 78 people (46 women) at the Katana market on prevention measures and signs of Ebola virus disease; educational talk with 19 motorcycle taxi drivers from the Kavumu/ZS center of Mitimurhesa on prevention measures and reporting of alerts; harmonization of the community relay briefing module.
- **Ituri:** 42 women representing three communes of the Bunia municipality were briefed on the various preventive measures for Ebola virus disease (EVD) at the market and in their living environments (emphasis was placed on infection prevention and control (IPC), particularly through proper handwashing and adherence to EDS practices); Results of analysis of feedback collected (Several complaints in the community related to the lack or poor availability of services promoted by the CREC (late EDS, poor response to alerts notified by community actors and the helpline, poor availability of decontamination teams, poor availability of care, especially nutritional, which is the basis for multiple evacuations); 7 mass awareness campaigns were organized in 7 different affected health zones on EVD preventive measures (1,210 people reached).

4.8 Logistics and Security

- Monitoring of construction work at the CTE, HGR Bunia (finishing work in progress), CME BUNIA, HGR Rwampara (CTE).
- Delivery of laboratory equipment to INRB/Beni, including 24 flash thermometers for the PNHF and 4 thermometers flash for HGR Katwa via ZS Katwa.
- Preparation of the input needs assessment for the pillars in the Katwa Health Zone and monitoring of maintenance vehicle in the area as well as the movement of the 4 vehicles provided by the IOM.
- Monitoring of the rehabilitation work of the CTE at the Katwa General Referral Hospital.

4.9. Security

- Raising awareness among the wives of military personnel in charge of compartments and their children on the general aspects of the MVE.

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the presence of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Awareness-raising of 822 people (**75 women, 45 men, 358 girls, 344 boys**) • Briefing of teachers in the Lita health zone on the PSEA (27 participants, including 22 men and 5 women).
- Awareness-raising on the prevention of sexual exploitation, abuse and harassment (PSEA) in the Miti Murhesa health zone under the theme: humanitarian aid is free and cannot be exchanged, sexual relations with children are prohibited, a total of 802 people including 67 women, 41 men, 357 girls and 337 boys.

5. MAIN CHALLENGES

Challenge No.	Impact	Action required
1. Reluctance to accept post-sampling mortem (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3. Weakness in contact tracing (overall rate 50.3%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5. Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6. Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action			Due date
		Responsible pillar	
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 43.2%) Surveillance		Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Catch-up plan for the 193 samples awaiting analysis in North Kivu	Laboratory	Day 2

9	North Kivu: discharge of the recovered case to Goma (Day 1), response to security incidents (ADF incursion), IPC training at the General Referral Hospital Virunga, CREC extension to 15,000 households	Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for IT/ECZS	Day 2

Briefing of EDS teams in the NIZI ZS, Ituri province.



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